

Steady — Handoff 10 follow-up: clinical questions and sign-off

Reference STEADY-CLINREV-2026-09-25-10F · Prepared 2026-09-25 · Follows STEADY-CLINREV-2026-09-24-10
· Generated from the app's registers by scripts/gen-h10-followup-worksheet.ts

Three parts, for three different people. Part A is for the two psychologists who signed the 24 September review. Part B is for a provider partner's clinical lead, when there is one. Part C is the founder's. Mark one option per item, add notes, initial, and sign at the end of your part.

Part A — For Rebecca Altschuler, PhD, and John Allen, PhD

Follow-ups to the 24 September review (STEADY-CLINREV-2026-09-24-10). Each item says what the app does today; mark one option or write your own. Nothing here changes a row you already signed unless you say so.

A1. Open questions

1. clinical.itq-between-visits

The second trauma questionnaire (the International Trauma Questionnaire) has no published past-week version that we know of. How often should members take it between visits?

What happens now: It is given once every thirty days, with its standard past-month wording.

- ☐ **Monthly, as written** recommended
Keep the published wording and give it once a month, next to the weekly checklist. *Then:* Nothing changes.
- ☐ **Weekly, with an adapted past-week wording**
Change its wording to ask about the past week. It would no longer be the published questionnaire, and its scores could not be compared with published ones. *Then:* The psychologists write and sign the adapted wording, then it opens every seven days.
- ☐ **Only at the start**
Ask it once when someone joins and leave between-visit tracking to the checklist. *Then:* It leaves the between-visit questionnaire list.
- ☐ **Something else** _____

Notes:

Initials _____

2. clinical.program-fit-pause-length

After the first eight questions stop someone because of thoughts of self-harm in the past month, or an unsafe situation now, how long before they may answer again?

What happens now: Twenty-four hours, as before. Answers a person cannot truthfully change — a diagnosis, a hospital stay in the past year, relying on substances, being under eighteen — now stay in place until someone from the care team has reviewed them and written down what they did; answering again no longer lifts them.

☐ **Until the question's own period has passed** recommended

The self-harm question asks about the past thirty days, so an honest answer cannot change for thirty days. A person from the care team can reopen it sooner after talking with them. *Then:* The pause becomes thirty days for that question, and stays twenty-four hours for an unsafe situation now.

☐ **Fourteen days**

The figure in the clinical rules document the safety rules were built from. *Then:* The pause becomes fourteen days for both questions.

☐ **Keep twenty-four hours**

A short pause, relying on honest answers the second time. *Then:* Nothing changes.

☐ **Something else** _____

Notes:

Initials _____

3. clinical.steadier-sleep-gates

The sleep program's four parts came with no level of distress or safety at which each one opens. The second part asks a member to get out of bed at night when they cannot sleep. Which levels should the parts use?

What happens now: The levels you already signed for the two night practices (row A14), chosen by the product owner on 25 September: parts one, three and four open on any day, even the hardest, like "After a bad dream"; part two opens only on a steadier day with distress at 7 or below, like "Back to rest". On other days it says "not today" and nothing is lost.

- ☐ **Keep the night-practice levels** recommended
What runs now. The part that asks a member to get up at night waits for a steadier day. *Then:* Nothing changes.
- ☐ **Open every part on any day**
Getting up after twenty minutes awake is safe enough to suggest even on the hardest days. *Then:* Part two opens at the same level as the others.
- ☐ **Hold the whole program for steadier days**
Treat it like the activity program, which waits for a steadier day throughout. *Then:* Every part needs a steadier day, with distress at 7 or below.
- ☐ **Something else** _____

Notes:

Initials _____

4. product.feelings-word-list

Two signed screens ask a member to pick from a feelings word list — Feeling and Relating's "Pick two words for how you feel right now", and the thought record's "What did you feel?" — but the signed content does not include a list. Should there be one, and which words?

What happens now: Members type one or two words of their own. Nothing is suggested, so nobody is steered toward a word that does not fit; the cost is that a member who cannot find a word gets no help finding one.

- ☐ **Add a short list the psychologists write** recommended
A plain list, including mixed and body-based words, reviewed like the rest of the pack. *Then:* The list appears once its review row is signed, with typing still allowed.
- ☐ **Keep typing only**
The member's own words, always. No list to steer them. *Then:* Nothing changes.
- ☐ **Something else** _____

Notes:

Initials _____

5. clinical.riding-strong-feelings-gates

The signed row says Riding Strong Feelings is gated "per underlying skills" without giving levels. Each part holds skills with different levels. When should a part open?

What happens now: A part opens when any of its skills would, and inside it each skill still shows only when its own level allows. So on a very hard day, parts one and two open with "Move it out" and "Find the room" — the skills made for that — and parts three and four wait for a steadier day.

☐ **When any of its skills would** recommended

What runs now. The program is there when feelings run high, showing only what fits the day. *Then:* Nothing changes.

☐ **Only when all of its skills would**

A part opens only when every skill in it is open, so the part is whole or not there. *Then:* Parts one and two wait for a steadier day, with distress at 7 or below.

☐ **Something else** _____

Notes:

Initials _____

6. clinical.thought-record-privacy

Members' thought records are signed as "private to the member". The strength of the feeling, before and after, is a number the care team could use to see whether the skill helps. Should those two numbers reach the care team, without the words?

What happens now: Nothing about a thought record reaches the care team: not the words, not the numbers, not that one was written. The program activities are different: their ratings do reach the care team's analytics.

☐ **Keep them fully private** recommended

The signed row read strictly: a thought record is the member's alone, numbers included. *Then:* Nothing changes.

☐ **Share the two numbers only**

The care team sees that a record was made and how strong the feeling was before and after, never the words. *Then:* The two numbers join the care team's analytics, the way program ratings do.

☐ **Something else** _____

Notes:

Initials _____

A2. Decisions the product owner made on 25 September — please confirm

7. clinical.sleep-entry-screen-retake answered by the product owner — please confirm

The sleep program asks three safety questions on joining; a yes to any leaves out the part about getting up at night. If someone leaves the program and joins again, should the questions be asked again?

Decided: Yes, after leaving and rejoining — the product owner's decision on 25 September, with the risk that a member could answer no the second time to get the left-out part back. For the psychologists to confirm at the next review.

☐ **Yes, after leaving and rejoining**

Things change: a health question can stop applying. Leaving clears the answers and joining again asks all three again. The risk: someone could answer no the second time only to get the left-out part back. *Then:* The earlier answer is kept, marked cleared, so the care team can see a change. A different answer the second time brings the part back.

☐ **No, the first answer stands**

Once a part is left out it stays out, unless someone from the care team reopens it. *Then:* Joining again skips the questions and keeps the part left out.

☐ **Something else** _____

Notes:

Initials _____

8. clinical.complex-path-program-before-review answered by the product owner — please confirm

Feeling and Relating is signed for the complex trauma readiness path only once that path's clinician review is done, but the app cannot record that review yet. What should members on that path see meanwhile?

Decided: Hide it until reviews exist — the product owner's decision on 25 September.

☐ **Hide it until reviews exist** recommended

Members on the complex trauma path do not see it, even if they are on the trauma path as well. Recording a review becomes its own piece of work. *Then:* Members on the trauma path alone see it; the complex path waits.

☐ **Build the review mark now**

A care-team action on the person record that opens the program for that member. *Then:* The complex path sees it once someone from the care team records the review.

☐ **Something else** _____

Notes:

Initials _____

A3. Wording and readings to confirm

9. Row KB_SELF_HOLD_REPLACES_BUTTERFLY (unsigned)

The self-hold that replaced the Butterfly Hug is confirmed by the Handoff 10 review (CV10_A01, 2026-09-24). This row covers what that row did not: made at the same time and for the same reason, two movement practices were reworded — the neck roll ('roll it slowly side to side') now rests the chin for a few breaths, and the rooting stance's 'sway forward and back, side to side' now settles weight into the heels and then evenly across both feet — and the calm-place lesson no longer says to pair the calm place with bilateral stimulation. Confirm these wordings, or mark needs-change.

☐ Approve

☐ Needs change (say what)

Notes:

Initials _____

10. "Marching in place" in the skill "Move it out" (S04, signed as A02/A03)

Step 1 reads: "Pick something your body can do safely: a brisk walk, marching in place, pushing against a wall." Marching alternates left and right in a steady rhythm, which is close to the bilateral shape v1 excludes. It was signed as written and ships as written. Should it stay, or be replaced (for example with "standing up and stretching")?

☐ Keep as written

☐ Replace (write the wording)

Notes:

Initials _____

11. How the 24 September sheet was read (1 of 3)

CV10_B02 asked the reviewers to choose the proposal, the knowledge-base default, or their own values, and was approved without a choice marked. It is read as approving the proposal, the only values written in the row: units 1 and 2 at stabilization, ceiling 6; units 3 and 4 at cautious, ceiling 6; Gentle items only at stabilization.

☐ That is what we meant

☐ We meant something else (say what)

Notes:

Initials _____

12. How the 24 September sheet was read (2 of 3)

CV10_A04 asked whether the companion should keep offering the cold-water skill at all, and was approved without an answer to that part. The skill stays out of the member library and the companion's behaviour is unchanged, which is the row's own default.

☐ That is what we meant

☐ We meant something else (say what)

Notes:

Initials _____

13. How the 24 September sheet was read (3 of 3)

CV10_C05 approves PROMIS Sleep Disturbance 8a as the candidate measure. No sleep measure is added until its licensed item text is supplied: the content pack does not contain it and it is not written from memory.

☐ That is what we meant

☐ We meant something else (say what)

Notes:

Initials _____

Part A sign-off

Rebecca Altschuler, PhD (AZ PSY-005804) _____ Date _____

John Allen, PhD (AZ PSY-002055) _____ Date _____

Part B — For the partner's clinical lead (Lane E, Phase 3)

The clinician-assigned lane was built ahead of its gate on the product owner's instruction (25 September) so a team can review and test it. It is not live: outside the demo it does not exist until these rows are approved. This part is for the clinical lead of the provider partner who will assign and review, not for Part A's reviewers.

B1. Rows

14. CV10_E01

Lane rules: never self-startable; active assignment required; assignments expire; distress 0 to 10 before and after; grounding, SOS, and clinician flag if after exceeds before by 3 or more, or exceeds 7.

What is built: Built and testable in the demo only. A practice is listed only for a person with a live assignment, which must have an end date; an expired or guessed link says "not available". Each start asks the existing safety engine (steady tier, no crisis today, no dissociation hold). Distress 0 to 10 before and after; a rise of 3 or more, or above 7, shows grounding and SOS and alerts the care team. Stop is on every step and is never counted against anyone.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

15. CV10_E02

Written exposure container: narrative stored encrypted, visible to member and assigning clinician only, fully excluded from the companion. Retention per partner policy.

What is built: "Written exposure (clinician-assigned)", about 30 minutes. No protocol text: the partner supplies it; the demo shows placeholders. Writing is encrypted, readable by the person and the assigning clinician only, and unreachable from the companion and every AI path (tested).

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

16. CV10_E03

CPT worksheet container: worksheets supplied by the partner under license; same lane rules.

What is built: "Worksheets (clinician-assigned)", about 20 minutes. Worksheet boxes the partner defines; the same handling as E02.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

17. CV10_E04

Imagery rehearsal container (nightmares): supplied by the partner; same lane rules and narrative handling as E02.

What is built: "Nightmare rehearsal (clinician-assigned)", about 20 minutes. The same handling as E02; no content until supplied.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

18. CV10_E05

Protocol source and fidelity: who supplies each protocol, under what license, and who is responsible for fidelity. No protocol content is written by Steady.

What is built: Nothing is written by Steady. Each practice records its content owner as the partner and cannot run outside the demo until the partner's licensed content is loaded.

- ☐ **Approve**
- ☐ **Approve with changes** (write them below)
- ☐ **Needs change**

Notes:

Initials _____

B2. Questions

19. partner.crisis-check-on-exposure-writing

For the partner's clinical lead: every piece of writing in the app is checked for crisis language before it is saved, and a match saves nothing and opens the crisis page. Exposure writing can describe past harm in words that match. Should the check run on it, and what should happen on a match?

What happens now: The check runs. A match saves none of that step's writing, ends the practice, alerts the care team without the words, and opens the crisis page. Earlier steps' writing stays saved.

- ☐ **Check it, keep the writing, flag the clinician** recommended
A match still opens the crisis page and alerts the care team, but the writing is kept for the assigning clinician. *Then:* Nothing written is lost; the clinician sees it with the flag.
- ☐ **Check it and save nothing, as now**
The same rule as everywhere else in the app. *Then:* Nothing changes.
- ☐ **Do not check exposure writing**
The assigning clinician reviews it; the distress ratings and flag still apply. *Then:* The check is skipped for writing steps in this lane only.
- ☐ **Something else** _____

Notes:

Initials _____

20. partner.who-reads-assigned-writing

For the partner's clinical lead: what a person writes in a clinician-assigned practice is readable by them and by the clinician who assigned it. Should anyone else on the care team be able to read it?

What happens now: Only the person and the assigning clinician. A colleague who opens it is told whose it is and shown nothing; the distress flag still reaches the care team's queue, without the words.

- ☐ **Assigning clinician only** recommended
As the handoff says: the person and the one clinician who gave them the practice. *Then:* Nothing changes.
- ☐ **Their whole care team**
Anyone caring for the person can read it, for continuity when the assigning clinician is away. *Then:* Every clinician on the person's care team can open the writing; each read is still recorded.
- ☐ **Something else** _____

Notes:

Initials _____

21. partner.assigned-writing-retention

For the partner's clinical lead: how long is writing from a clinician-assigned practice kept, and may the person delete it themselves?

What happens now: Kept until the person's account is deleted. The person cannot delete it on their own, because it may form part of the partner's clinical record; the handoff leaves this to the partner's record policy.

☐ **The partner's record policy** recommended

Kept for the period the partner's records must be kept, then removed. *Then:* A retention period is set for this lane from the partner's policy.

☐ **The person can delete it**

Like everything else they write in Steady, with two taps. *Then:* A delete control appears beside their writing; the clinician's view shows it was deleted.

☐ **Something else** _____

Notes:

Initials _____

22. partner.distress-flag-levels

For the partner's clinical lead: the signed lane rule flags the assigning clinician when distress after a practice is 3 or more above distress before, or above 7. Are those the right levels, and should a practice the person stopped be flagged by the same rule?

What happens now: Those levels, applied whether the practice was finished or stopped, when a rating after was given. A person who stops may skip the rating, and then nothing is flagged.

☐ **Keep them, finished or stopped** recommended

The rule as written, applied to every run with a rating. *Then:* Nothing changes.

☐ **Also flag any stop**

A stop is not a failure, but the clinician may want to know it happened. *Then:* Every stopped practice sends a gentle note to the clinician's queue.

☐ **Different levels**

Name the rise, and the level after the practice, that should reach the clinician. *Then:* The two levels change.

☐ **Something else** _____

Notes:

Initials _____

23. partner.lane-titles-and-lengths

For the partner's clinical lead: the three practices need names a person reads and a length. The handoff names one, "Written exposure (clinician-assigned)". Are "Worksheets (clinician-assigned)" and "Nightmare rehearsal (clinician-assigned)", at about 30, 20 and 20 minutes, right?

What happens now: Those names and lengths, marked as drafts; no method is named to the person.

☐ **Keep them** recommended

Plain names that describe the activity without naming a method. *Then:* Nothing changes.

☐ **The partner's own names**

Use whatever the partner's protocols call them for patients. *Then:* The names and lengths change to the partner's.

☐ **Something else** _____

Notes:

Initials _____

Part B sign-off

Partner clinical lead (name, credentials, licence) _____ Date _____

Part C — For you (founder, with counsel): not clinical

Items no clinician can sign. Tick when done and file the document where noted.

24. F01 — sleep measure licence

Sleep measure licensing: confirm the terms for using the chosen sleep measure inside a commercial product. You signed F01; the licence itself is not in the repository. Until its item text is supplied, the sleep program adds no measure (C05).

☐ Licence filed (where)

☐ Not yet

Notes:

Initials _____

25. F04 — audio rights

Audio rights: written rights for narrator recordings and soundscapes. You signed F04; the agreements are not in the repository, and no recordings exist yet. Package 11 (audio) waits on them.

☐ Agreements filed (where)

☐ Not yet

Notes:

Initials _____

26. Phase 3 — before any real person uses the clinician-assigned lane

Handoff 10 §6 lists four conditions and the lane was built before them on your instruction: (1) Handoff 03's activation gate, including the repo delta note and Handoff 02's scheduler; (2) a signed provider partner whose clinicians assign and review; (3) licensed protocol content from that partner; (4) HIPAA posture, a BAA, and the Postgres cutover. Part B's sign-off is also required.

☐ Acknowledged: none of the four is met today

☐ Some are met (say which)

Notes:

Initials _____

Part C sign-off

Founder _____ Date _____